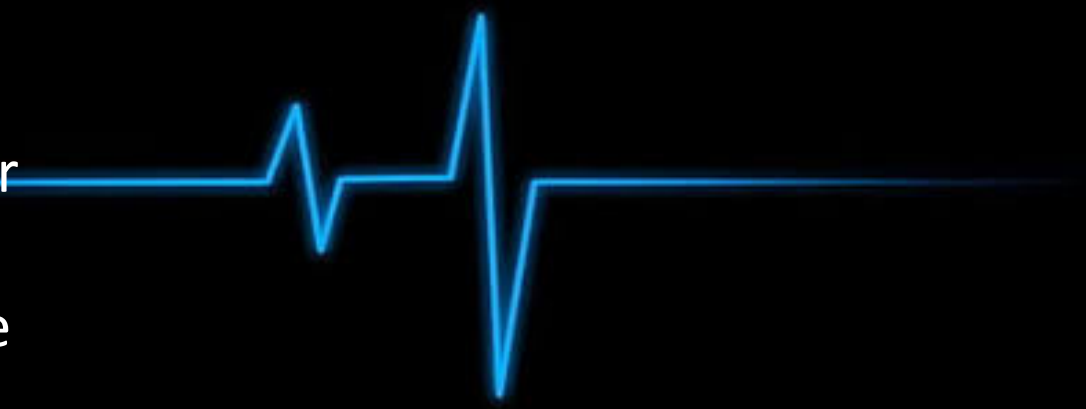
Case Presentation

E. Sukumar, JIPMER



Patient Details

- Name: Mr.xyz
- Age : 46year
- Sex: Male
- Occupation: Lawyer
- Address: Cuddalore



Chief complaints

- Breathlessness for 5 months
- Palpitations for 2months
- Cough for 3weeks



History of Presenting Complaints

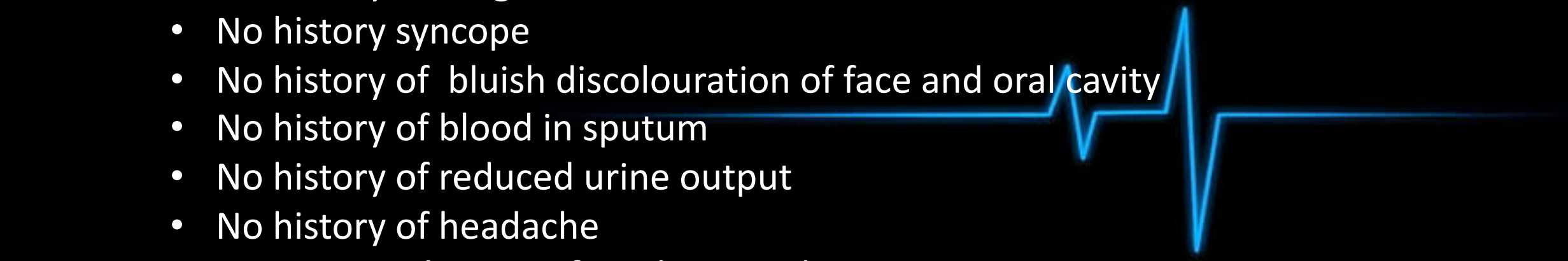
- The patient was apparently asymptomatic 5 months ago, when he developed breathlessness.
- The breathlessness was insidious in onset, progressing from NYHA grade II to NYHA grade IV over last 3 months and associated with palpitations.
- The breathlessness increased on lying down and exertion . The patient also had complaints of night time awakenings in last 3months. Decreased on sitting and it was associated with use of accessory muscle. No seasonal variations was noted. Daily activities were affected in the past 2weeks.

History of Presenting Complaints

- The palpitations were exertional and irregular, noted along with breathlessness for past 2 months. It is associated with sweating and sometimes felt during sleep.
- The cough started 3weeks back. It was progressive with whitish, non-foul smelling sputum (approx 5ml/day) and not blood stained.
- The cough aggravated on lying down with no diurnal variation and seasonal variations.
- Lately the cough has increased in frequency and limits the activity of the patient.

History of Presenting Complaints

- No history of fever
- No history of fatigue
- No history syncope
- No history of bluish discolouration of face and oral cavity
- No history of blood in sputum
- No history of reduced urine output
- No history of headache
- No previous history of similar complaints



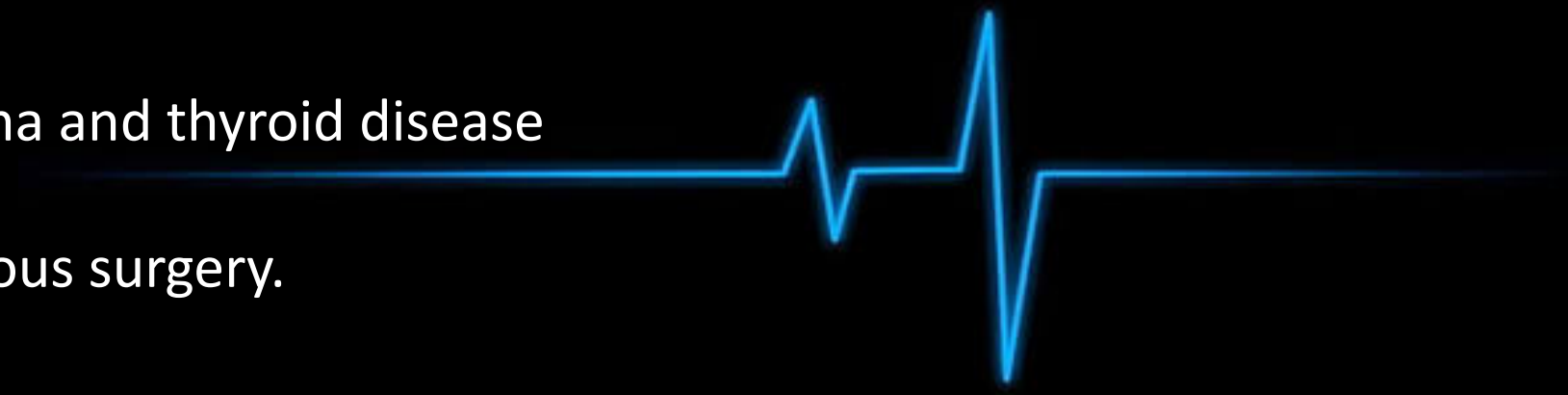
Past history

- When the patient was 16yrs, there was an episode of low grade fever with severe joint pain in the wrist followed by knee for 3days. The joint pain was associated with redness and decreased mobility.He was treated in local GH, and the illness resolved completely after 7days of antibiotics was given.

- 2months ago, patient had low grade intermittent fever for 1week which resolved on medications and had history of coughing out clotted bloods(early morning). After a few days, he started having frequent episodes of fever with breathlessness which relives on antibiotic medication.

Past history

- Not a known case of Diabetes Mellitus and hypertension.
- No history of Tuberculosis or contact with TB.
- No history of asthma and thyroid disease
- No history of previous surgery.



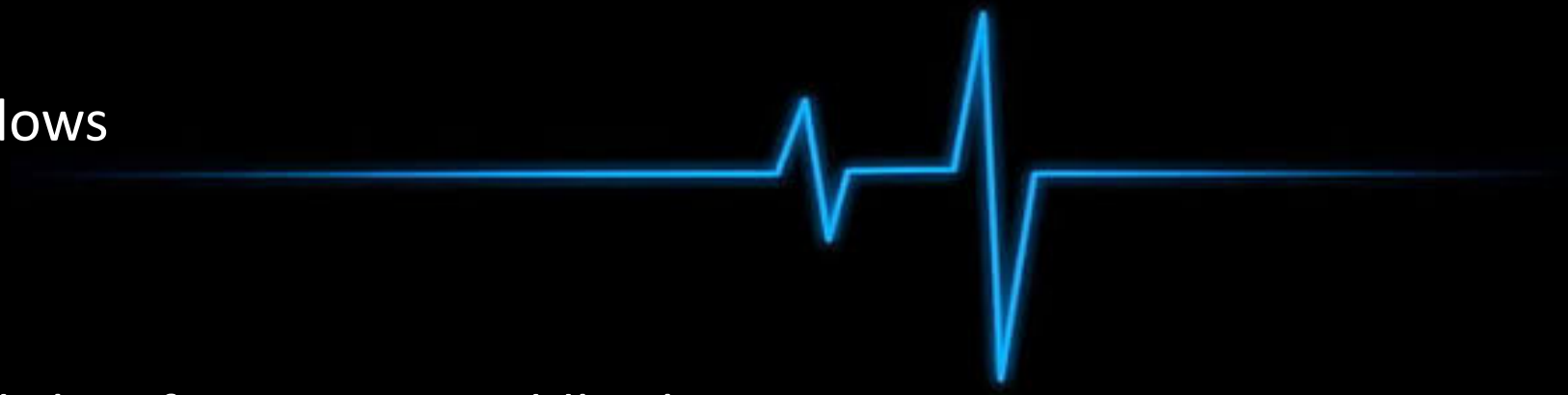
Personal and Family history

- Decreased appetite and normal sleep
- Normal Bowel and bladder movements
- No known addiction
- Family history not significant



Socioeconomic history

- Pakka house
- 3 members live in the house
- 2 rooms and 2 windows
- Income : ₹9,000
- Modified BG Prasad classification => middle class



Summary

- A 46 year old male belonging to middle class (modified BG Prasad) presented with complaints of progressive breathlessness and orthopnea. He also has history of irregular palpitations and progressive non productive cough worsening in past 2weeks. No history of fever, chest pain, syncope or hemoptysis.

- Probable diagnosis:

  - Rheumatic heart disease
  - Degenerative valvular heart disease
  - Dilated cardiomyopathy
  - Parenchymal lung disease

General examination

- The patient was conscious oriented to time, place and person. Moderately built and nourished.

- He had mild pallor,
- No icterus
- No cyanosis
- Grade 2 clubbing was present
- No generalised lymphadenopathy and
- Bilateral pitting pedal edema

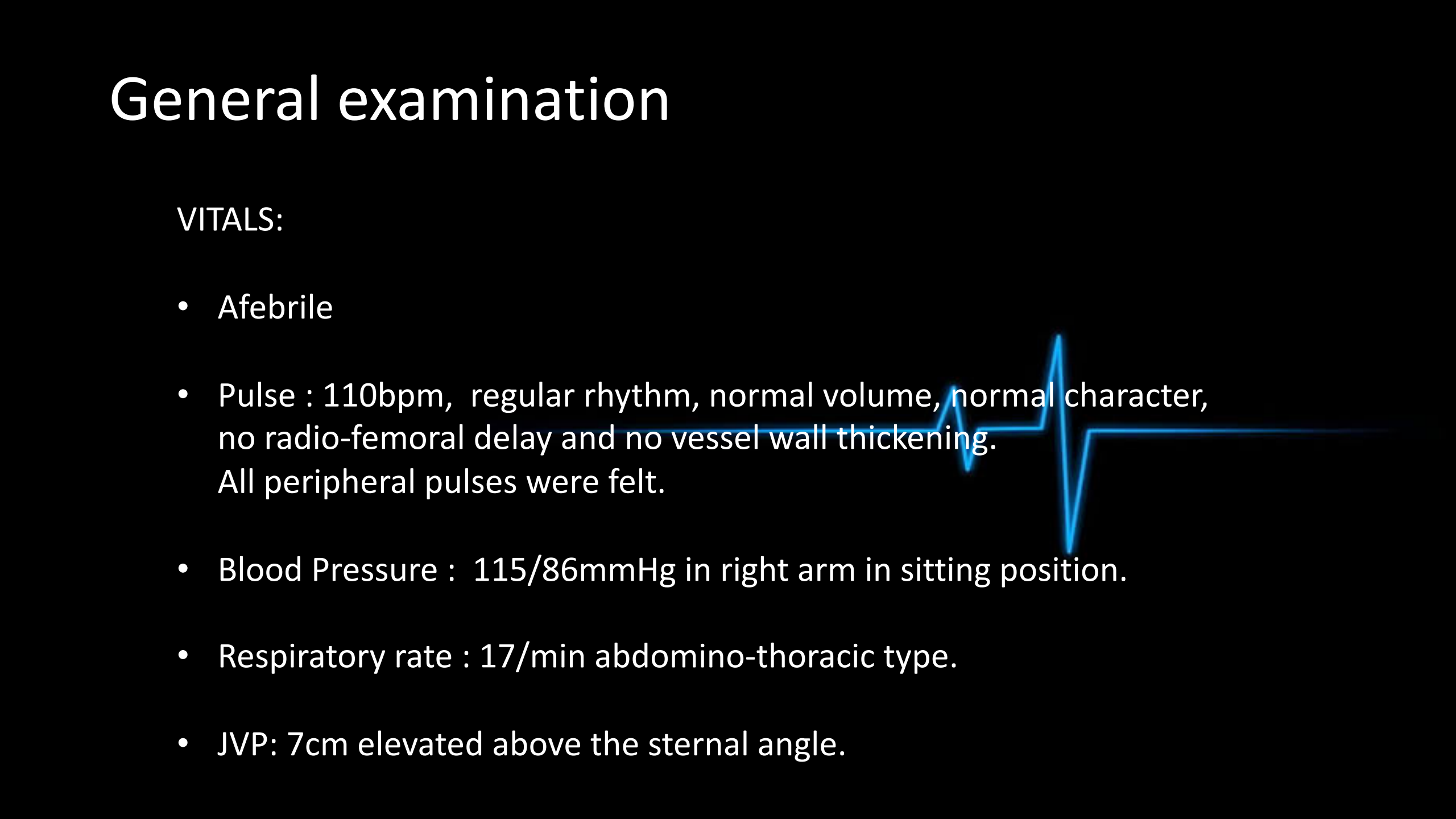


- No spinal anamolies

General examination

VITALS:

- Afebrile
- Pulse : 110bpm, regular rhythm, normal volume, normal character, no radio-femoral delay and no vessel wall thickening.
  All peripheral pulses were felt.
- Blood Pressure : 115/86mmHg in right arm in sitting position.
- Respiratory rate : 17/min abdomino-thoracic type.
- JVP: 7cm elevated above the sternal angle.



Inspection

- Trachea appears in midline
- Chest wall symmetrical
- Presence of precordial bulge
- Apex beat seen in left 6th intercostal space ,medial to mid clavicular line.
- No other visible pulsation seen
- No dilated veins
- No scars and sinuses.

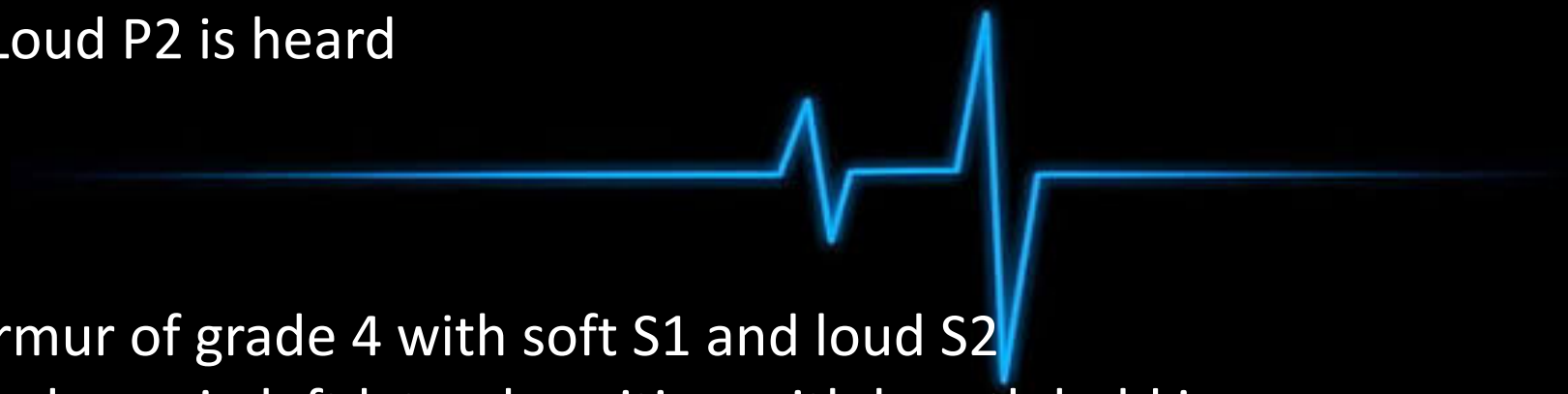
Palpation

- Inspectors findings were confirmed in palpation.
- Trachea is in midline.
- Apex beat is hyperdynamic, felt on 6th intercostal space on the midclavicular line.
- Parasternal heave of grade 4 on left side is felt
- Palpable systolic thrill is present
- Palpable P2 is present.
- Epigastric pulsation hitting the tip of index finger is noted.



Auscultation

- Mitral area: soft S1 is heard.
- Tricuspid area: S1 and soft s2 is heard
- Aortic area: S1 and loud S2 is heard
- Pulmonary area: Loud P2 is heard



- Murmur

  A pansystolic murmur of grade 4 with soft S1 and loud S2 best heard in mitral area in left lateral position with breath held in expiration , radiating to left axilla and back with no variation in respiration is heard using the diaphragm of the stethoscopes. The murmur also increases on recumbent position and decreases on standing.

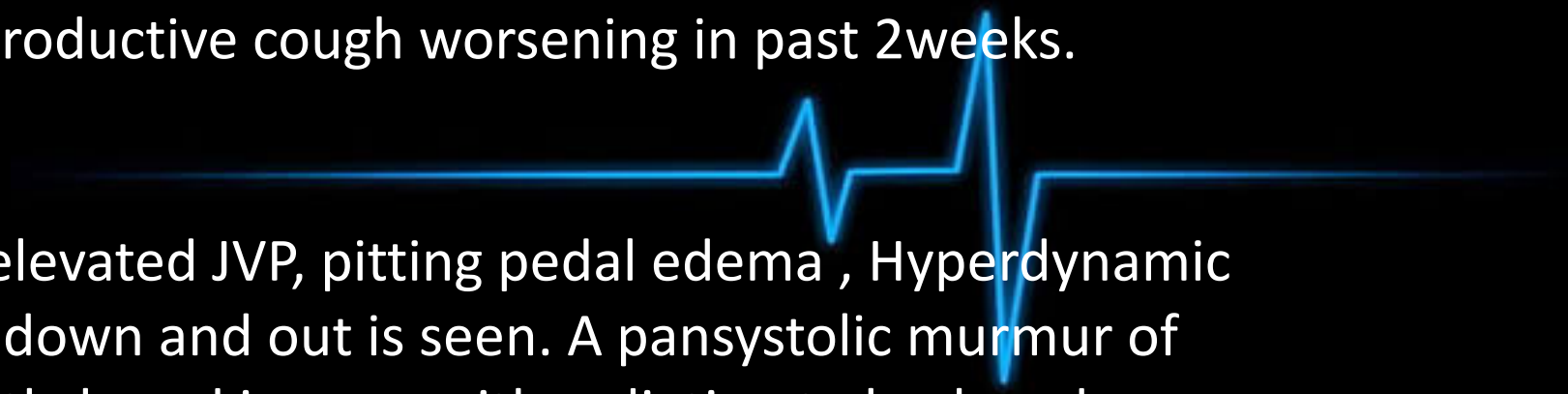
Other system

- RS
  - Normal vesicular breath sounds heard
- CNS
  - No focal neurological deficits found
- GIT
  - Mild Tender hepatomegaly was present.



Summary

- A 46 year old male belonging to middle class (modified BG Prasad) presented with complaints of progressive breathlessness and orthopnea. He also has history of irregular palpitations and progressive non productive cough worsening in past 2weeks.



- On examination, elevated JVP, pitting pedal edema , Hyperdynamic apex beat shifted down and out is seen. A pansystolic murmur of grade 4 prominently heard in apex with radiation to back and a parasternal heaving is noted.

- The findings are suggestive of acquired valvular heart disease - Mitral regurgitation.

Diagnosis

- Acquired valvular heart disease of probable rheumatic etiology - Mitral regurgitation with pulmonary hypertension and atrial fibrillation with evidence of congestive heart failure without signs of infective endocarditis.

